



POLICY

Medication: Prescribing, Administration and Management in the Community

Scope (Staff):	North Metropolitan Health Service Mental Health (MH) Staff: Community Medical and Nursing
Scope (Area):	NMHS MH Areas: Community Clinics

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1. Aim

To provide information and guidance on the prescribing, ordering, supply, administration and storage of medications. This refers to all disciplines that are authorised to prescribe, dispense, administer or manage medications.

2. Background

The policy will be followed in accordance with the [Western Australian Legislation - Medicines and Poisons Act 2014](#), [Western Australian Legislation - Medicines and Poisons Regulations 2016](#), [WA Medicines Handling Policy \(MP 0139/20\)](#) and the [Reporting of Schedule 4 Restricted and Schedule 8 Medicines Discrepancies Policy \(MP0103/19\)](#)

3. Risk

Failure to comply with this policy introduces risk that patients will not have access to medication in an appropriate and timely manner; patients may suffer adverse medication events; the NMHS MH will not be compliant with the legislative requirements regarding medicines management; and NMHS MH will experience unnecessary financial expenditure.

Governance & Accountability	☒	Policy & Legislative Compliance	☒
Staff Safety & Wellness	☒	Quality of Patient Care	☒

4. Definitions

Medication: A medication is defined as any substance or medication used for therapeutic or diagnostic purposes.

Dispense: Dispense in relation to a medicine means supplying the medicine in accordance with a prescription duly given by a Medical Practitioner. **Only a Pharmacist or Medical Practitioner may dispense.**

Administration: Administration is defined as the giving of a medication to a patient by a Registered Nurse or a medication competent Enrolled Nurse to be taken or used immediately. Administration is not deemed to be dispensing.

Prescribe: An authorised person, at NMHS MH this means a medical officer, can issue a prescription for a medicine.

5. Principles

5.1 Prescribing of Medications

- Prescribing will be in accordance with the [NMHS MH Medication-Formulary & Non-Formulary Prescribing Policy](#).
- Prescribers must use the Best Practice® prescribing software to record and generate prescriptions and related documentation including payment vouchers and Clozapine Blood

Count Record Form (Form 5). Where Best Practice® software is unavailable, paper prescriptions may be used.

- Best Practice® prescribing software will not be used to generate prescriptions or other documents for persons other than patients of the service.
- Prescriptions must be issued in accordance with the requirements of the Pharmaceutical Benefits Scheme (PBS), wherever possible.

5.2 Consumer Medication Information

In accordance with the [WA Medication Review Policy MP 0104/19](#):

- Prescribers will offer up to date Consumer Medication Information (CMI) or other written information and discuss compliance and discontinuation issues when a medication is prescribed and reviewed.
- This information will be accessible and readable for all patients, including the culturally and linguistically diverse community and Aboriginal Australians if possible.
- This information will be accompanied by a verbal explanation of medication treatment options, benefits, associated risks and side-effects.
- A brief summary of the above actions must be documented in the patient's Integrated Progress Notes.
- CMIs for psychotropic medications and some medications used to treat side-effects of psychotropics are available in different forms on the [Choice and Medication](#) website.
- Other medication information leaflets are available on the [Formulary One](#) site or MIMS (via [Psychiatric Services Library](#)).

If applicable, the information will also be provided to the patient's carer or family members.

5.3 Prescribing during normal hours of operation

- Medical staff must generate prescriptions for patients as appropriate.
- Prescriptions may be written up to the maximum PBS quantity and repeats if appropriate.
- Depot medications may be prescribed up to the maximum PBS quantity and repeats.
- Benzodiazepines and non-benzodiazepine hypnotics may only be prescribed for 10 days continuously or if PRN for a maximum of 30 doses in every 3-month prescription cycle. Where clinically necessary to provide ongoing prescribing or supply greater than described above the prescriber must discuss with the Head of Clinical Services (HoCS) and document in the patient's Integrated Progress Notes the clinical circumstances for ongoing or greater supply.
- Payment vouchers may be provided for the following:
 - non-PBS medication where the patient pays a co-payment and the service is billed for the remaining cost

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- where dose administration aids or interval dispensing is deemed necessary for clinical reasons
 - full exemption from payment in accordance with [NMHS MH Medication: Exemption from Patient Prescription Co-payment](#)
 - a voucher is not required for depot medication when ordered via the [Patient's Depot Injections Ordered and Received Form](#).

5.4 Prescribing outside of normal business hours of operation

This includes extended hours and after-hours emergency service.

- The on-call psychiatrist must call a community pharmacy to dispense an appropriate quantity of emergency medication. The on-call psychiatrist must provide the pharmacy with a valid prescription within 72 hours of the telephone order.
- The on-call psychiatrist must send the pharmacy a [voucher](#) for submission to NMHS MH pharmacy for re-imbursement with the prescription.
- The Pharmacy Guild provides a regularly updated list of pharmacies and their opening hours at www.findapharmacy.com.au.
- The medication can be collected from the pharmacy by the patient or patient's carer; or if this is not possible, by the Assessment and Treatment Team (ATT) staff member.
- The ATT/on-call staff member should record the details of the medications supplied in the PSOLIS service event.

5.5 High Risk Patients

- Consideration should be given to risk minimisation strategies when prescribing for patients assessed as high risk of self-harm or suicide.
- Examples of strategies includes prescribing small amounts of medication, organising staged supply of medication or management of medication by carers rather than the patient.

6. Administration of Medication

6.1 General principles

- All medications must only be administered by a Registered Nurse or medication Competent Enrolled nurse on the instruction of medical staff.
- The **6 Rights** of safe medication administration must apply:
 - Right patient – (NMHSMH AW Identification of Patients Policy)
 - Right medicine



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- Right dose
 - Right time
 - Right route
 - Right documentation

Also check if the patient has any ADRs or allergies with the medication prescribed.

- Be aware of the following aspects of the medication:
 - Generic name of the medication
 - Actions and therapeutic use
 - Side effects
 - Specific precautions
- When checking a medication prescription, nurses must always be aware of the following aspects of the medication before administration:
 - Generic name of the medication
 - Action and therapeutic purpose
 - Adverse reactions/ allergies
 - Interactions
 - Contraindications
 - Usual dosage
 - Frequency
 - Times for administration
 - Route of administration
 - Specific precautions.
- The preparation of a medication dose including an injection must only occur immediately prior to administration.
- Any error/near-miss medication administration incident must be reported immediately to the Clinical Nurse Specialist (CNS) or the designated senior Nurse and relevant medical staff. Medication errors must be reported on the Clinical Incident System (Datix CIMS) and reported to the NMHS MH Chief Pharmacist via the system 3rd Party comment module. The incident must be investigated by the CNS or designated senior Nurse. The nominated person, carer, family member or personal support person must be notified in the case of a medication error (refer to [WA Mental Health Act 2014](#)).
- The CNS or delegate will ensure that the medication recording system is audited on a monthly basis to ensure that doses taken are being recorded correctly and record keeping adheres to legislative requirements.
- The HoCS and Coordinator of Nursing, Community MHS will review the audit results on a monthly basis and ensure corrective action is taken if required.



6.2 Verbal Orders

- Verbal orders for medication may be provided by medical staff to a Registered Nurse (RN) under circumstances where a single dose of medication is to be administered to a patient attending a clinic.
- The RN receiving the verbal order will:
 - Repeat the order to the Prescriber
 - Record the prescription on the [CMHS Record of Imprest Medication](#)
- The RN receiving the verbal order must include in the record:
 - Patient's name and UMRN
 - Name of medication
 - Time and date to be given
 - The dose to be administered
 - Route of administration
 - Prescriber's name
 - Name and signature of the CMHN taking the order.
- The prescriber will record the verbal prescription in the patient's Integrated Progress Notes. The record will include date, drug name; dose administered and reason for administration.
- The final decision to accept a verbal medication prescription is a decision made by the RN.

7. Adverse Medication Reactions (ADR)

Any adverse medication reaction must be managed in accordance with the [WA Clinical Alert \(MedAlert\) Policy](#) and the [NMHS MH Management of Clinical Alerts Policy](#).

It must be reported in the patient's Integrated Progress Notes, included on the patient's PSOLIS and Best Practice® Alerts and the Clinical Alert PMR 01 filed in the front of the patient's clinical records. If the reaction is life-threatening a Clinical Alert Notification form PMR 02 should be completed.

Any new adverse medication reactions occurring during a clinic admission should be reported to the Therapeutic Goods Administration (TGA) using the online process. Refer to the [TGA On-line Adverse Event](#).

8. Storage and Security of Medications

- All medications must be stored in a secure locked medication cupboard or fridge in accordance with the manufacturer's storage recommendations. It is the responsibility of the Duty Nurse (RN) or delegate to ensure these are always locked.
- A key register must be kept for all keys accessing the impost medication, patients' own medications and depot injections (including the medication fridge).

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- Medication keys will only be issued to nursing staff, who must complete the register when receiving or relinquishing the keys.
 - The CNS or delegate will be responsible for maintaining the key register and issuing medication keys, and checking compliance.
 - Imprest medication, patients' own medication and depot injections must not be stored in the same cupboard.
 - A signature register must be maintained for all nursing staff.

8.1 Imprest Medication

- Community Mental Health Clinics (CMHC) may keep impost medication in accordance with [Appendix 1](#).
- Clinic impost medication is to be used by a Medical Officer or Registered Nurse to administer to patients attending the clinic. Imprest medication will not be given to patients to take home.
- Imprest medication must only be administered on the authority of a Medical Officer.
- Documentation in the progress notes and Best Practice that impost medication was administered must occur.
- Imprest medication will be recorded in the [CMHS Record of Imprest Medication](#) by the receiving RN.
- The record will indicate the:
 - date of the transaction
 - name of the person supplying and/or patient receiving the medication
 - patient's Unit Medical Record Number (UMRN)
 - name, strength, form and quantity of medication received or administered
 - name and signature of the staff member performing the transaction.
- Imprest medication can be ordered by a RN via the NMHS MH Pharmacy using the Imprest order form. The order must be signed by a member of medical staff.
- Medication kept at community clinics should be checked weekly by RN or medication Competent Nurse to ensure it hasn't expired, not damaged or no longer required.
- Supply of impost medication must not be sought from community pharmacies or other hospitals.
- Sample packs of medication provided by pharmaceutical companies must not be accepted or used in CMHC. [NMHS Medication: Pharmaceutical Representatives – Access to NMHS MH Staff](#)

8.2 Patient's Own Medication (POM)

- All POMs must be recorded by a registered nurse on receipt in the clinic and on delivery to the patient on the [CMHC Patient's Own Medication Register](#).
- The record must indicate the date received, the patients name and UMRN, the name and signature of the Registered Nurse (RN) receiving, the date handed out and to whom, and the name and signature of the RN issuing the medication. If medications are in dose administration aids, dose administration aids can be entered as a description and the number of packs, in place of medication names and strengths.
- Any Schedule 4 Restricted (S4R) medication supplied to a patient is stored and recorded as for other POMs. A Schedule 4 Restricted Register is not required.

8.3 Excess or unnecessary medications removed at a patient's residence

- It is important when visiting patients at their residence or at a clinic appointment for staff to discuss with patients if they have any excess or unnecessary medication.
- If appropriate, patients should be encouraged to take excess or unnecessary medication to their local pharmacy for disposal or to relinquish it to the staff members for disposal at a local pharmacy.
- If excess or unnecessary medications are identified during the visit at the patient's residence, the action taken should be to first communicate this and any non-compliance with medication to the medical team who will make the decision on the next step/s. This communication and the outcome of the communication must be comprehensively documented in the patients' Integrated Progress Notes.
- If a patient is requested to relinquish excess or unnecessary medication, but is unwilling to do so, this information **must be documented** in the patient's Integrated Progress Notes and communicated to the medical team so that a risk assessment of the possible danger can be conducted and appropriate action be taken. This may warrant management of the patient as a High Risk patient in accordance with clause 5.5.

9. Antipsychotic Long-Acting Injections (LAI)

- Referred to also as depots.
- Each clinic must identify a RN who is responsible for the ordering and management of depot injections. They must familiarise all staff with the process to ensure cover is available for absences due to illness, leave or days off.
- The identified RN is responsible for the weekly ordering of all depot medication required from the local MOU pharmacy using [COMMUNITY MENTAL HEALTH CLINICS: Patient's Depot Injections Ordered and Received Form](#) or electronic equivalent.



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- Depot administration must be recorded on the patient's depot medication chart. This chart is to be filed in the patient's Integrated Progress Notes when completed.
 - Unused stock is not to be allocated to another patient, nor should the dispensed label be altered in any way.
 - Unwanted or expired depot medications should be taken to the local community pharmacy for destruction.
 - Refer also to [NMHS MH Medication: Intramuscular Administration of Long-Acting Antipsychotics Guideline](#).

10. Returning Unwanted/ Expired Stock

- Any medication including imprest medication that is no longer required or expired upon notification by the identified RN should be taken to the local community pharmacy for destruction.
- Imprest medication taken to the local community pharmacy for destruction should be recorded on the [CMHS Record of Imprest Medication form](#), with the reason for destruction.
- This must be stored in a clearly labelled separate shelf/box in the locked medication cupboard (or fridge) until collected.
- Patients should be directed to their community pharmacy to dispose of their unwanted oral medication wherever practical due to storage restrictions within the community clinics.

11. Schedule 8 Medications

Schedule 8 medications must not be kept in CMHC.

12. Compliance and Evaluation

- This policy will be audited by NMHS MH Pharmacy. Compliance with prescribing requirements will be reported and monitored via the Pharmaceutical Services Group.
- Medication errors reported via Datix CIMS will be audited and reported to the NNMHS MH Drugs and Therapeutics Committee and MH Management Committee.

Related internal policies, procedures and guidelines

[NMHS MH Medication-Formulary & Non-Formulary Prescribing Policy](#)

[NMHS MH Medication: Intramuscular Administration of Long-Acting Antipsychotics Guideline](#)

References

[Western Australian Medicines and Poisons Act \(2014\)](#)

[Western Australian Medicines and Poisons Regulations \(2016\)](#)

[Reporting of Schedule 4 Restricted and Schedule 8 Medicines Discrepancies Policy MP 0103/19](#)

[WA Medicines Handling Policy MP 0139/20](#)

[WA Health Statewide Medicines Formulary Policy MP 0077/18](#)

[WA Medication Review Policy MP 0104/19](#)

Useful resources

[WA Health Choice and Medicines Printable Patient Medication Information Leaflets](#)

[CMHC Patient's Own Medication Register](#)

[NMHS MH COMMUNITY MENTAL HEALTH CLINICS: Patient's Depot Injections Ordered and Received Form](#)

[NMHS MH Psychiatric Services Library](#)

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Policy Contact	NMHS MH Chief Pharmacist				
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National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 6: Consumers ☒ Std 7: Carers		☐ Std 8: Governance, leadership and management ☒ Std 10: Delivery of Care		
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers,				
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Document Version Control			
Date	Version	Author	Notes
01/08/2017	2.0	Chief Pharmacist, Chair NMHS MH DTC	Inclusion of imprest medication
18/05/18	2.1	A/Policy Officer	HealthPoint hyperlinks repaired. Link to ATT voucher repaired
14/06/18	2.2	PSG Meeting	Inclusion of clarifying statement on Benzodiazepine prescribing. Plus inclusion of clarifying statement on depot medication script length and no voucher needed when ordering with depot order form.
14/06/2019	2.3	Chief Pharmacist, Chair NMHS MH DTC	Major Amendment: Addition of statement on provision of consumer medicines information, updated hyperlinks, updated template. Addition of statement on addressing hoarding of medications. Removal of hyperlink of rescinded OD 0377/12 and replaced with MP0103/19.
10/07/2019	2.4	Chief Pharmacist, Chair NMHS MH DTC	Major Amendment: Inclusion of requirement to restrict use of Best Practice prescribing software to patient care provision.
19/05/2020	2.5	Chief Pharmacist, Chair NMHS MH DTC	Minor Amendment: update broken links.
	3.0	Chief Pharmacist, NMHS MH	New template updating information. Removing WA policies that have been rescinded. Inclusion of additional information regarding high risk patients and consumer medication information.
20/09/2022	4.0	A/Chief Pharmacist, Chair DTC	New template updating information. Removing WA policies that have been rescinded, updating links to current policies. Inclusion of additional information regarding high risk patients and consumer medication information.
03/03/2023	4.1	A/Chief Pharmacist, NMHS MH	Addition of information regarding removal of excess or unnecessary medication from a patient's residence, as per L. Philpott, Assistant State Solicitor

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative ISD Number: 2090.

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Appendix 1. List of Approved Medications for Community Mental Health Clinics

PROGRAM	MEDICATION	QUANTITY
Adult	Olanzapine 5mg	1 x 28 wafers (or oral disintegrating tablets (ODT))
Older Adult	NIL	
Youth	NIL	